LAKESIDE ORTHOPEDICS
Spine · Sports · Joint · 2 Lakeside Dr, Riverton · (555) 010-6500 · Fax (555) 010-6501

Alana Reyes, MD
Orthopaedic Spine

ORTHOPEDIC CONSULTATION

Date of service 03/05/2026 (Thu) · 10:30 AM · New patient

PATIENT	DOB	MRN
DOE, JOHN	08/14/1991	LKO-218840
REFERRING	DATE OF INJURY	PAYER
Nina Patel, PA-C, Riverton Urgent Care	02/06/2026	Harbor Point MedPay HP-26-7741

History of present illness

34-year-old right-handed warehouse shift lead, restrained front-seat passenger in a vehicle struck on the passenger side on 02/06/2026. Neck stiffness at the scene (EMS evaluated; transport declined). Neck pain worsened over the next days; right thumb and index finger numbness from 02/18/2026. Seen at Riverton Urgent Care 02/20/2026 (cervical strain, r/o radiculopathy; cyclobenzaprine, naproxen). Chiropractic and PT at Riverton Spine & Rehab since 02/24/2026, with partial improvement in neck pain but persistent arm symptoms. Today neck pain 6/10, right arm tingling along the lateral forearm into the thumb and index finger several times a day, worse with extension and looking over the right shoulder. No gait change, no bowel or bladder symptoms, no dropping objects.

PMH: none. **Prior spine:** 2021 chiropractic care, 3 visits, low back only; no prior neck pain or treatment. **Meds:** naproxen 500 mg BID, cyclobenzaprine 10 mg HS PRN. **Allergies:** NKDA. **Social:** non-smoker.

Examination

- **Cervical:** right paraspinal and trapezius tenderness; no midline step-off. Flexion 40°, extension 30° (reproduces right arm paresthesia), rotation R 50° / L 65°.
- **Provocative:** Spurling's test **positive on the right** (paresthesia into the right thumb); negative on the left. Shoulder abduction relief sign positive right. Hoffmann negative bilaterally.
- **Sensory: right C6 dermatomal paresthesia** — decreased light touch and pinprick over the lateral forearm, thumb and index finger. C5, C7, C8 intact.
- **Motor:** 5/5 throughout except right wrist extensors 4+/5 and right biceps 5-/5. **Reflexes:** right brachioradialis 1+ (left 2+); biceps, triceps 2+ symmetric.
- **Shoulders:** full painless ROM, impingement signs negative.

Assessment

1. Cervical radiculopathy, right C6 distribution (M54.12), following motor vehicle collision 02/06/2026 — clinical picture suggests a C5-C6 disc injury. 2. Cervical sprain/strain (S13.4XXA), improving with conservative care.

Plan

1. **MRI cervical spine without contrast** — ordered; Summit Imaging Center, 88 Summit Pkwy (scheduled 03/13/2026).
2. Stop naproxen; start **meloxicam 15 mg** PO daily with food, #30, 1 refill. Continue cyclobenzaprine at bedtime PRN.
3. Continue chiropractic/PT at Riverton Spine & Rehab 2–3×/week; avoid end-range extension and heavy overhead lifting.
4. Work: full duty with no lifting over 30 lb overhead until reassessed.
5. Return after MRI to review results. Sooner for weakness or spreading numbness.

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FOLLOW-UP VISIT

Date of service 03/27/2026 (Fri) · 9:15 AM · Established patient

PATIENT	DOB	MRN
DOE, JOHN	08/14/1991	LKO-218840
LAST VISIT	DATE OF INJURY	PAYER
03/05/2026 — consult; MRI ordered	02/06/2026	Harbor Point MedPay HP-26-7741

Interval history

Returns to review the cervical MRI. Continuing chiropractic/PT 2–3×/week; neck pain improved to 4/10 and rotation better. Right thumb and index finger numbness persists 2–3 times a day and wakes him at night. Meloxicam helps the neck but not the arm symptoms. Working full shifts with difficulty on overhead tasks.

Imaging reviewed

MRI cervical spine without contrast, Summit Imaging Center, 03/13/2026 (Helen Park, MD). Images and report reviewed personally: **C5-C6 right paracentral disc herniation** contacting the ventral right C6 nerve root, with mild right foraminal narrowing. No cord signal change. Radiologist's impression notes no significant degenerative change at the adjacent levels. Findings correlate with the right C6 radicular exam.

Examination

- Rotation R 60° / L 70°; extension 35°, still reproduces right arm paresthesia.
- Spurling's positive right. Decreased light touch right C6 dermatome, unchanged. Right wrist extensors 4+/5; brachioradialis R 1+.
- No myelopathic signs; Hoffmann negative; gait normal.

Assessment

C5-C6 right paracentral disc herniation with right C6 radiculopathy (M50.121), post-traumatic onset after the 02/06/2026 collision; persistent radicular symptoms despite 4+ weeks of conservative care.

Plan

1. **Referral to pain management** — Crescent Pain & Spine, Dr. Omar Haddad, 41 Crescent Ct — for evaluation and **cervical epidural steroid injection** (right C5-C6 target). Referral and records faxed today.
2. Start **gabapentin 300 mg** 1 cap PO at bedtime, may increase to twice daily for arm symptoms; #90, 2 refills. Continue meloxicam 15 mg daily.
3. Continue chiropractic/PT. No surgical indication at this time; surgical consultation only if progressive weakness.
4. Return after the injection, or in 10–12 weeks for reassessment of maximum medical improvement.

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cc: Riverton Spine & Rehab (Mark Ellison, DC; Sara Lin, DPT) · Crescent Pain & Spine (referral)